

Policy on Infection Prevention and Control in Support Services

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Infection Prevention and Control Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

STATUTORY NOTE: Section 5.4 of this policy (biomedical waste) is designated statute P2. Those provisions follow the Biomedical Waste Management Rules applicable at the time of implementation. The accreditation-specific guidance in all other sections does not carry statute status.

Statement of intent

«Hospital Name» maintains engineering controls to prevent infection, uses a validated infection-risk assessment tool before any construction or renovation, follows documented housekeeping procedures across all areas, handles biomedical waste as per statutory provisions, and adheres to defined laundry/linen and kitchen sanitation processes — regardless of whether these services are in-house or outsourced.

1. Purpose

This policy explains how «Hospital Name» implements infection prevention and control processes in support services: engineering controls, construction/renovation infection risk management, housekeeping, biomedical waste, laundry and linen, and kitchen sanitation.

Clinical IPC processes, HAI-prevention bundles, and IPC surveillance are covered in separate hospital policies.

2. Scope

This policy applies to housekeeping staff, BMW handlers, laundry staff, kitchen and dietary staff, engineering/maintenance teams, and the IPC Officer at «Hospital Name», including those performing outsourced services.

3. Policy standards

«Hospital Name» maintains engineering controls including patient-care area design, OT air quality, water supply and HVAC maintenance. A validated infection-risk assessment tool is used before any construction or renovation; no works begin in a patient-care area without an approved infection-risk plan. Housekeeping is addressed at all levels of the organisation, with regular mandatory cleaning and documented procedures for terminal cleaning, blood/body-fluid cleanup and isolation rooms; dusting inside clinical areas is prohibited. Biomedical waste is segregated in colour-coded containers as per statutory provisions, handled with appropriate PPE, stored per statutory requirements, and handed to an authorised vendor. Laundry and linen management follows a defined process with clean linen always separated from dirty. Kitchen and dietary services adhere to statutory requirements, implement sanitation measures, and ensure food is safely prepared, handled, stored and distributed.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not start construction or renovation in a patient-care area until the infection-risk plan for that work has been completed and the IPC Officer has agreed the controls.
- Do not handle biomedical waste without the required colour-coded segregation and appropriate PPE.
- Do not perform any dusting inside clinical areas.
- Do not use common disinfectants without an established dilution protocol based on national/international guidelines.
- Do not store biomedical waste outside statutory requirements, and do not hand it to anyone other than an authorised vendor.
- Do not perform routine environmental microbiological sampling of OT and high-risk areas, do not conduct periodic fumigation, and do not fog unless commissioning a new OT, following modification/renovation of an OT, or when there is a significant increase in SSI with environmental evidence.
- Do not mix clean linen with dirty linen at any point.

5. What we do

5.1 Maintain engineering controls to prevent infection

Engineering controls address patient-care area design — including optimum spacing between beds — OT design and zoning, air quality, and water supply. The organisation follows NABH

guidelines on OT air-conditioning. HVAC plant and equipment, air-conditioning ducts and filters, air-handling units, and cooling towers are maintained regularly; filters are replaced on schedule. Water-supply sources, systems, and water-quality testing are included in the engineering controls. Fungal colonisation prevention is addressed. Those requirements related to facilities management are covered in the hospital's other policies.

5.2 Plan and implement infection controls for construction and renovation

Before any construction or renovation in or adjacent to a patient-care area, a validated infection-risk assessment tool — such as the Infection Control Risk Assessment (ICRA) — is used to identify the risk of infection. An infection prevention plan covering architectural segregation, traffic-flow controls, and materials management is developed and reviewed by the IPC team. Construction or renovation does not begin until the plan is complete and the IPC Officer has agreed the controls. «Hospital Name» considers IPC from the design stage when building new facilities. Surveillance of demolition, construction and repair areas is conducted as part of the IPC surveillance programme.

5.3 Follow housekeeping procedures

Housekeeping is addressed at all levels of the organisation — wards, OT, public areas including toilets, and corridors. Regular cleaning to remove visible dirt and dust is mandatory. The environment, fixtures, fomites, furniture, furnishings, equipment, and surfaces are cleaned as applicable. Common disinfectants are identified per national/international guidelines; dilution protocols are established and followed. Written procedures exist and are followed for terminal cleaning, blood and body-fluid cleanup, and isolation rooms. Dusting of any sort inside clinical areas is avoided. The effectiveness of housekeeping is monitored regularly through outcome parameters rather than only frequency verification — see also the IPC surveillance policy.

5.4 Handle biomedical waste appropriately and safely

Biomedical waste from patient-care areas is properly segregated and collected in colour-coded bags and containers as per the statutory provisions of the applicable Biomedical Waste Management Rules. BMW is handled with appropriate PPE. BMW is stored in accordance with statutory provisions. BMW is handed over to an authorised vendor for transport to the site of treatment and disposal. Monitoring of the BMW management programme is done by members of the IPC committee/team as per statutory requirements. These obligations follow the Biomedical Waste Management Rules applicable at the time of implementation; «Hospital Name» ensures its practices remain current with any amendments to those Rules.

5.5 Follow laundry and linen management processes

Linen management follows a defined process whether the service is in-house or outsourced. «Hospital Name» documents the change-of-linen procedure. A defined process covers handling linen in patient-care units, during transport to the laundry, and inside the laundry. The process for storage and distribution of clean linen is defined and followed. Clean linen is separated from dirty linen at all times. Where laundry is outsourced, adequate IPC controls are established to ensure compliance.

5.6 Follow kitchen sanitation and food-handling requirements

Kitchen sanitation and food-handling requirements apply whether the dietary service is in-house or outsourced. «Hospital Name» adheres to all statutory requirements, including screening of kitchen workers and food handlers. Kitchen sanitation measures are implemented to prevent cross-contamination. Food is prepared, handled, stored and distributed safely. Dietary services are designed with no criss-cross of traffic and with activities flowing in sequence. Hygienic conditions are maintained throughout.

6. Stop-work authority

Do not handle biomedical waste without the required colour-coded segregation and PPE.

Do not start construction or renovation in a patient-care area until the infection-risk plan for that work has been completed and the Infection Prevention and Control Officer has agreed the controls.

Stop-work applies to the waste-handling act and to the construction/renovation start. Emergency repair that cannot wait follows the documented emergency-works IPC controls and is recorded.

The person who stops tells the Infection Prevention and Control Officer and the Medical Superintendent the same shift. Refusing unsafe waste handling or unplanned works is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring support-service IPC processes are resourced and implemented; receives stop-work escalations the same shift for construction and renovation triggers.
Infection Prevention and Control Officer	Owns implementation of this policy; approves infection-risk plans before construction or renovation; receives stop-work escalations the same shift; monitors BMW management programme compliance.
Engineering / Maintenance In-Charge	Ensures engineering controls, HVAC and water systems are maintained; submits planned construction and renovation works to the IPC team for ICRA before commencement.
Housekeeping In-Charge	Ensures housekeeping procedures are followed in all areas; reports deviations to the IPC Officer.
BMW / Waste Management Officer	Ensures colour-coded segregation, PPE use, statutory storage, and authorised vendor handover; maintains BMW monitoring

	records.
Dietary / Kitchen In-Charge	Ensures statutory requirements, sanitation measures, and safe food-handling practices are followed, whether in-house or outsourced.
Quality Coordinator	Audits this policy; holds training records, staff acknowledgements, and stop-work event logs.

8. Quality monitoring

Monitoring area	What is monitored
Engineering controls	HVAC and water-system maintenance records; OT air-quality records; engineering-round records.
Construction/renovation ICRA	Completed ICRA tool and approved infection-risk plan on file before works start; IPC Officer sign-off documented.
Housekeeping effectiveness	Outcome parameters monitored regularly; terminal-clean records for isolation rooms; no-dusting policy confirmed in clinical areas.
BMW management	Segregation audit records; PPE compliance; statutory storage records; authorised vendor handover log; statutory monitoring records.
Laundry and linen	Linen management process records; clean/dirty separation confirmed in IPC round.
Kitchen sanitation	Kitchen worker screening records; statutory compliance records; sanitation round records.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

Housekeeping staff, BMW handlers, laundry staff, kitchen staff, and engineering/maintenance staff shall be familiar with their respective IPC obligations under this policy, including the stop-work authority in Section 6.

I have read the Policy on Infection Prevention and Control in Support Services of «Hospital Name». I will follow the processes described, including the stop-work authority in Section 6.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to the IPC Officer, housekeeping staff, BMW handlers, laundry staff, kitchen staff, engineering/maintenance teams, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
BMW	Biomedical Waste
CAPA	Corrective and Preventive Action
HEPA	High-Efficiency Particulate Air
HVAC	Heating, Ventilation and Air Conditioning
ICRA	Infection Control Risk Assessment
IPC	Infection Prevention and Control
IPCO	Infection Prevention and Control Officer
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OT	Operation Theatre
PPE	Personal Protective Equipment
SSI	Surgical Site Infection

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
IPC.4.a	Commitment*	Sections 3 and 5.1 address engineering controls for patient-care area design, OT air quality, water supply, and HVAC/duct/filter/cooling-tower maintenance.
IPC.4.b	Commitment*	Sections 3, 5.2 and 6 address the validated infection-risk assessment (ICRA or equivalent) and approved infection-risk plan before any construction/renovation; stop-work trigger at construction/renovation start.
IPC.4.c	CORE*	Sections 3 and 5.3 address mandatory regular cleaning, disinfectant identification with dilution protocols, terminal cleaning/blood

		spill/isolation room procedures, and the prohibition on dusting in clinical areas.
IPC.4.d	CORE	Sections 3, 5.4 and 6 address colour-coded BMW segregation per statutory provisions, PPE, statutory storage, authorised vendor handover, and monitoring per statutory requirements; stop-work trigger for BMW without segregation/PPE. Statute P2 designated.
IPC.4.e	Commitment*	Sections 3 and 5.5 address the documented linen management process: in-unit handling, transport, laundry, storage and distribution, clean/dirty separation at all times, and outsourcing controls.
IPC.4.f	Commitment*	Sections 3 and 5.6 address kitchen sanitation and food handling: statutory requirements including worker screening, sanitation measures, safe food preparation/handling/storage/distribution, and traffic/sequence design.

13. Required Records/Evidence Checklist

Engineering controls — IPC.4.a (Commitment*)

- Engineering controls record (bed layout/spacing, OT zoning, water and air systems) and annual review record.
- HVAC and duct/filter/cooling-tower maintenance schedule and service logs.
- Water-quality test results.

Construction/renovation ICRA — IPC.4.b (Commitment*)

- Completed ICRA tool or equivalent for each construction/renovation project, before works begin.
- Written infection-risk plan covering architectural segregation, traffic flow, and material controls, with IPC Officer sign-off.
- Records confirming no works started before plan approval.

Housekeeping — IPC.4.c (CORE*)

- Written housekeeping procedures by area (ward, OT, public areas) including cleaning frequency, products, dilution protocols, and special procedures for terminal cleaning, blood/body-fluid cleanup, and isolation rooms.
- Cleaning records and outcome-monitoring records.
- IPC round records confirming no dusting inside clinical areas.

Biomedical waste — IPC.4.d (CORE, Statute P2)

- BMW segregation audit records confirming colour-coded bags and containers per statutory provisions.
- PPE compliance records for BMW handlers.
- BMW storage records confirming statutory requirements met.
- Authorised vendor agreement and handover logs.
- BMW monitoring records as per statutory requirements.
- Stop-work event log for BMW-trigger events.

Laundry and linen — IPC.4.e (Commitment*)

- Written linen management process covering handling in units, transport, laundry, storage and distribution of clean linen.
- Change-of-linen documentation.
- Outsourcing IPC control records, where applicable.

Kitchen sanitation — IPC.4.f (Commitment*)

- Kitchen worker and food-handler screening records (statutory requirement).
- Kitchen sanitation procedure and daily sanitation check records.
- IPC round record that includes the dietary/kitchen area.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. IPC.4.
2. Ministry of Environment, Forest and Climate Change, Government of India. (2016). <https://clip.cpcb.gov.in/index.php/rule/bio-medical-waste-management-rules-2016/> Bio-Medical Waste Management Rules, 2016 (as amended).

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

Several requirements in this document are statutory rather than advisory — in particular those arising under the Biomedical Waste Management Rules (as amended), Government of India. Statutory requirements change, and State authorities impose additional or stricter conditions.

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